NCCP 00483 · VERSION 13A

Your treatment, explained

Unresectable or metastatic melanoma in adults; the team confirms the source-specific eligibility and previous treatment. · NCCP 00483 v13a · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Nivolumab · every 2 weeks · advanced melanoma

Reference treatment cycle

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
The overall treatment plan
Your 14-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Understanding your diagnosis and treatment goal
Your medicines and the overall plan
Taking treatment and preparing for visits
Tests: what is required and what each result means
What to expect between visits
Skin checks, sun protection and optional photographs
Pregnancy, fertility and protecting others
Questions, decisions and practical support
Follow-up and your continuing record
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

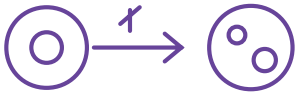
Medicines in this regimen

Nivolumab helps immune cells act against melanoma that cannot be completely removed or has spread. The aim is disease control; benefit and side effects are reviewed for you.

PD-1 CHECKPOINT IMMUNOTHERAPY

Nivolumab

Opdivo



Releases a PD-1 immune brake

Nivolumab blocks PD-1, a signal that can reduce T-cell activity. This can help immune cells act against melanoma, but it can also allow inflammation in healthy organs. It is not conventional chemotherapy, and side effects can occur after treatment stops.

PD-1 blocked → Immune activity increases → Healthy organs can inflame

What to watch for

Carry your immunotherapy alert card and report new symptoms early, including after treatment ends.

Sources: nccp · nccn-patient

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Hormones / head / eyes

Nivolumab

New severe headache, visual change, unusual tiredness, dizziness, thirst or frequent urination may signal immune effects and needs prompt advice.

3 Heart / muscles

Nivolumab

Chest pain, palpitations, marked muscle weakness or swallowing difficulty can be serious immune effects. Seek urgent assessment.

5 Kidneys / urine

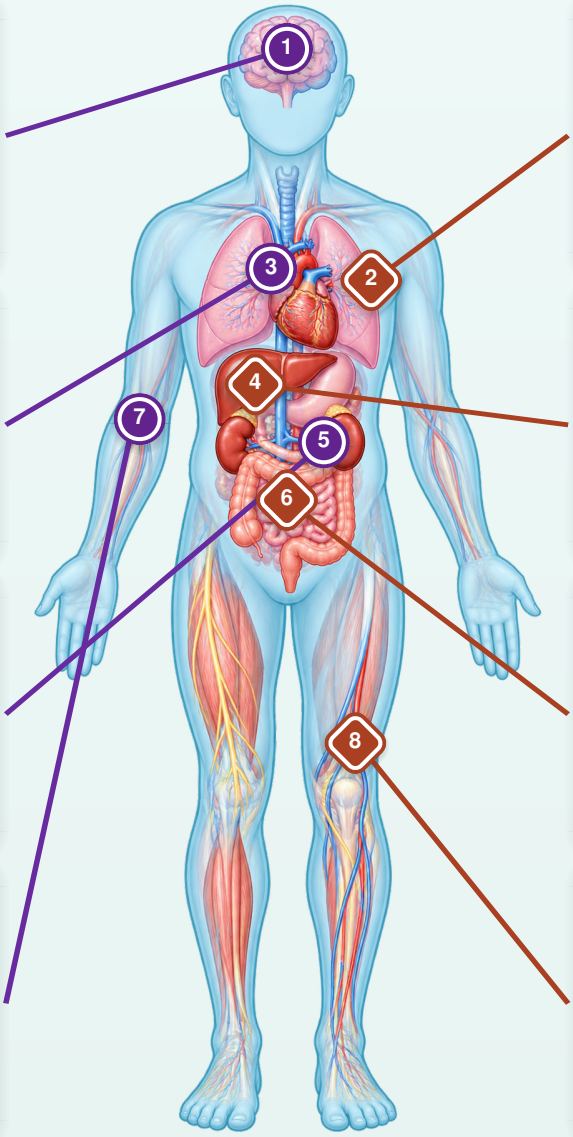
Nivolumab

Kidney inflammation may be detected on blood tests. Report reduced urine, swelling, blood in urine or a marked change from normal.

7 Skin / mouth

Nivolumab

Report rash or itching early. Blistering, peeling, mouth or genital sores, or rash with fever needs urgent assessment.



2 Lungs / breathing

Nivolumab

New cough or breathlessness may reflect lung inflammation or another serious cause. Call promptly; severe breathing difficulty is an emergency.

4 Liver

Nivolumab

Inflammation may first show in blood tests. Report yellow eyes or skin, dark urine, unusual itching or upper-abdominal pain.

6 Bowels / abdomen

Nivolumab

Report increased stool frequency, diarrhoea, blood or mucus, or abdominal pain early. Severe pain, black stools or dehydration needs urgent help.

8 Nerves / strength

Nivolumab

New tingling, weakness, unsteadiness, facial change or trouble swallowing needs prompt review. Sudden weakness or speech difficulty is an emergency.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

The overall treatment plan

EVERY 2 WEEKS

Ongoing treatment and review

Day 1 every 14 days | **Duration individually reviewed**

Nivolumab is given as 240 mg by IV infusion over 30 minutes OR 600 mg by subcutaneous injection over 3–5 minutes, on Day 1 every 2 weeks. Allow extra time for assessment, preparation and observation. The team selects one formulation. A subcutaneous injection is given into the thigh or abdomen, alternating sites and avoiding damaged skin, scars or moles. IV and subcutaneous doses are not interchangeable milligram for milligram. Treatment continues until progression or unacceptable toxicity, with individual benefit and safety review.

The team confirms treatment dates, recovery and each safety decision. A planned treatment count does not authorise the next dose.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00483 v13a: current regimen source · NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

THE WHOLE TREATMENT JOURNEY

Nivolumab · every 2 weeks · advanced melanoma: treatment and checks

Unresectable or metastatic melanoma in adults; the team confirms the source-specific eligibility and previous treatment.

Nivolumab every 2 weeks. Treatment continues until progression or unacceptable toxicity, with individual benefit and safety review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP	Blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune conditions, transplants, previous immune reactions and all current medicines. BRAF tumour status is checked for the advanced or adjuvant melanoma indication.	Blood count, kidney/ liver tests and glucose before every 2-week cycle. Thyroid function is checked once a month and when clinically indicated. Cortisol and other tests are added when clinically indicated. Report new symptoms at every visit and between visits; normal blood results do not rule out immune toxicity.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue immune-toxicity monitoring for at least five months after the last dose, and longer when advised. Keep any prescribed steroid taper, hormone replacement and specialist follow-up. Scans follow your individual disease plan.
Cancer, skin and daily function NCCP	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/ lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.
Medicines and continuing precautions NCCP	Review all medicines and the pregnancy/ fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Discuss fertility before starting. Use effective contraception during treatment and for at least 5 months after the last dose. Tell the team promptly about a possible pregnancy. Discuss breastfeeding restrictions with the team before treatment; do not breastfeed without their advice. Do not stop prescribed steroids or hormone replacement yourself.

What each check is for

1

Ongoing treatment and review

Every 2 weeks

Nivolumab helps immune cells act against melanoma that cannot be completely removed or has spread. The aim is disease control; benefit and side effects are reviewed for you.

Ask / act: Confirm the current phase, prescription, tests and support plan.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.41

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Nivolumab every 2 weeks. Treatment continues until progression or unacceptable toxicity, with individual benefit and safety review.

Treatment timing: displayed days 1–14

Treatment or check	Days in this window	What happens
Nivolumab treatment	Day 1	Nivolumab is given as 240 mg by IV infusion over 30 minutes OR 600 mg by subcutaneous injection over 3–5 minutes, on Day 1 every 2 weeks. Allow extra time for assessment, preparation and observation. The team selects one formulation. A subcutaneous injection is given into the thigh or abdomen, alternating sites and avoiding damaged skin, scars or moles. IV and subcutaneous doses are not interchangeable milligram for milligram.
Symptoms between visits	Days 1–14	Immune effects can begin at any time, including after treatment stops.
Prepare next-cycle review	Day 14	Arrange the required bloods and symptom review before the next dose. Thyroid checks follow their own interval.

Your schedule: Nivolumab every 2 weeks. Treatment continues until progression or unacceptable toxicity, with individual benefit and safety review.

Tests between visits: Blood count, kidney/liver tests and glucose before every 2-week cycle. Thyroid function is checked once a month and when clinically indicated. Cortisol and other tests are added when clinically indicated. Report new symptoms at every visit and between visits; normal blood results do not rule out immune toxicity.

Changing the interval: If changing from two-weekly to four-weekly nivolumab, the first four-weekly dose is two weeks after the last two-weekly dose. For the reverse change, the first two-weekly dose is four weeks after the last four-weekly dose. The team confirms the formulation and dates.

Held or missed treatment: If a visit is missed or treatment is held, contact the unit for a new plan. Do not count a missed appointment as a dose received. An interval change, operation or new phase does not erase earlier immune toxicity or a permanent-stop decision.

Safety decisions: Routine dose reductions are not recommended for these immunotherapies. The clinician may hold or stop treatment and treat the immune effect.

> **Why these bands stay broad:** This calendar shows the pattern. Your team supplies actual dates, prescribed doses and any hold or restart instructions. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00483 v13a: current regimen source ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Complete timing and symptom notes

Day 1: Treatment visit

Nivolumab is given as 240 mg by IV infusion over 30 minutes OR 600 mg by subcutaneous injection over 3–5 minutes, on Day 1 every 2 weeks. Allow extra time for assessment, preparation and observation. The team selects one formulation. A subcutaneous injection is given into the thigh or abdomen, alternating sites and avoiding damaged skin, scars or moles. IV and subcutaneous doses are not interchangeable milligram for milligram. Report symptoms and previous reactions before treatment.

Day 2–14: Between visits

Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy. Do not wait for the next treatment to report a possible immune effect.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

Nivolumab is given as 240 mg by IV infusion over 30 minutes OR 600 mg by subcutaneous injection over 3–5 minutes, on Day 1 every 2 weeks. Allow extra time for assessment, preparation and observation. The team selects one formulation. A subcutaneous injection is given into the thigh or abdomen, alternating sites and avoiding damaged skin, scars or moles. IV and subcutaneous doses are not interchangeable milligram for milligram.

STEP 2

Missed, vomited or held treatment

If a visit is missed or treatment is held, contact the unit for a new plan. Do not count a missed appointment as a dose received. An interval change, operation or new phase does not erase earlier immune toxicity or a permanent-stop decision.

STEP 3

Appointments and supply

Arrange dates, medicine supply, transport and any needed interpreter or carer support. Ask whether bloods are taken in the oncology unit or elsewhere and how the result reaches the prescriber.

STEP 4

Paper copies and contact information

Keep this complete handbook, the three-page summary and the wallet card together. The QR code is optional. Ask the team to add your actual contacts and print a revised plan when treatment changes.

Sources: NCCP 00483 v13a: current regimen source. Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 **An individual symptom plan**

Ask which anti-sickness or other symptom medicines are prescribed, when to take them and when to call if they do not help. Do not start a borrowed medicine or alter treatment to manage a side effect yourself.

2 **Your urgent contacts**

Keep your actual oncology, out-of-hours and pharmacy contacts on the wallet card. Example numbers on a template must be replaced before you use it.

3 Pregnancy and fertility precautions

Discuss fertility before starting. Use effective contraception during treatment and for at least 5 months after the last dose. Tell the team promptly about a possible pregnancy. Discuss breastfeeding restrictions with the team before treatment; do not breastfeed without their advice. Do not stop prescribed steroids or hormone replacement yourself.

4 Eating, activity and practical help

Report falling intake, weight loss, pain or fatigue early. The team can tailor nutrition, activity, wound, psychological and practical support to your needs.

5 Steroids and hormone replacement

Some immune effects need steroids or lasting hormone replacement. Follow the exact prescription and taper. Carry the relevant alert information, and do not stop or reduce these medicines yourself when anticancer treatment is held or completed.

Sources: nccp · nccn-patient

TREATMENT REFERENCE · 01

Understanding your diagnosis and treatment goal

Use these questions with your own pathology report and agreed treatment plan.

What the diagnosis says

Keep the pathology summary, original skin site, stage and details of any lymph-node involvement together. Breslow thickness and ulceration describe features of the original melanoma; they are different from the stage. If the diagnosis arose in the eye or a mucosal surface, ask whether this cutaneous-melanoma information applies.

Sources: nccn-patient

BRAF and other tumour tests

A BRAF V600 result helps decide whether the BRAF/MEK targeted medicines as a separate treatment option from the immunotherapy described in this guide are suitable. This is usually a test of the tumour, not proof of an inherited family condition. Keep the actual result and test date; ask about genetic counselling only when your personal or family history makes that relevant.

Sources: nccn-patient

Surgery, radiotherapy and medicine have different roles

Record what surgery removed and whether the present medicine is postoperative treatment to lower recurrence risk or treatment of cancer that remains or has spread. Radiotherapy, stereotactic treatment or another operation may have a separate role; their timing and side effects need their own plan.

Sources: nccn-patient

Stage and response are different

Stage describes the extent of melanoma at diagnosis or a later reassessment. A response assessment asks how known cancer has changed with treatment. In the adjuvant setting there may be no visible tumour to measure: surveillance looks for recurrence while safety checks look for medicine effects.

Sources: nccn-patient

A new skin change still needs review

Continue to report a new or changing mole, a non-healing lesion, a new lump or an unexplained persistent symptom. Do not assume every new lesion is melanoma, and do not use a photograph or a good response at one site to rule out a problem elsewhere.

Sources: nccn-patient

TREATMENT REFERENCE · 02

Your medicines and the overall plan

Nivolumab helps immune cells act against melanoma that cannot be completely removed or has spread. The aim is disease control; benefit and side effects are reviewed for you.

The schedule in this setting

Nivolumab every 2 weeks. Treatment continues until progression or unacceptable toxicity, with individual benefit and safety review.

Sources: nccp

How the treatment works

Nivolumab blocks PD-1, a signal that can reduce T-cell activity. This can help immune cells act against melanoma, but it can also allow inflammation in healthy organs. It is not conventional chemotherapy, and side effects can occur after treatment stops.

Sources: nccp

The role of earlier or later treatment

Record surgery, radiotherapy and other anticancer medicines separately, including dates and the specialist responsible. This schedule does not automatically include or authorise another treatment. Ask whether any overlap changes tests, symptoms to report or the timing of appointments.

Sources: nccn-patient

Pauses and stopping

A safety pause is an active treatment decision. Record which medicine is paused, the reason, the tests or recovery needed and who may authorise restarting. Do not restart from an old prescription or reset the history when a new phase begins.

Sources: nccp

TREATMENT REFERENCE · 03

Taking treatment and preparing for visits

Keep the written medicine plan beside this guide; it records your own dose and dates.

Taking or receiving the medicine

Nivolumab is given as 240 mg by IV infusion over 30 minutes OR 600 mg by subcutaneous injection over 3–5 minutes, on Day 1 every 2 weeks. Allow extra time for assessment, preparation and observation. The team selects one formulation. A subcutaneous injection is given into the thigh or abdomen, alternating sites and avoiding damaged skin, scars or moles. IV and subcutaneous doses are not interchangeable milligram for milligram.

Sources: nccp

Missed, vomited or held treatment

If a visit is missed or treatment is held, contact the unit for a new plan. Do not count a missed appointment as a dose received. An interval change, operation or new phase does not erase earlier immune toxicity or a permanent-stop decision.

Sources: nccp

Other medicines and supplements

Show the pharmacist a current list of prescriptions, over-the-counter medicines, vitamins, herbal products and supplements. Include anticoagulants, heart medicines, steroids and recent antibiotics. Check before adding, stopping or changing anything; an apparently harmless product can alter treatment or confuse a symptom assessment.

Sources: nccp

Practical preparation

Arrange medicine supply, transport, interpreter or carer support before it is needed. Bring the alert card, symptom record and questions. Ask which appointments require someone to accompany you and how to obtain urgent advice outside normal hours.

Sources: nccn-patient

TREATMENT REFERENCE · 04

Tests: what is required and what each result means

Safety tests, tumour tests and response scans answer different questions.

Before treatment

Blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune conditions, transplants, previous immune reactions and all current medicines. BRAF tumour status is checked for the advanced or adjuvant melanoma indication.

Sources: nccp

During treatment

Blood count, kidney/liver tests and glucose before every 2-week cycle. Thyroid function is checked once a month and when clinically indicated. Cortisol and other tests are added when clinically indicated. Report new symptoms at every visit and between visits; normal blood results do not rule out immune toxicity.

Sources: nccp

Scans and tumour markers

Ask the team to write the next assessment and who will arrange it. Bloods used to monitor kidneys, liver, hormones or muscle are not automatically measures of cancer response. There is no universal tumour-marker blood schedule for these skin-cancer regimens; a scan or extra blood test is requested for a specific clinical reason.

Sources: nccp

After a pause or finish

Continue immune-toxicity monitoring for at least five months after the last dose, and longer when advised. Keep any prescribed steroid taper, hormone replacement and specialist follow-up. Scans follow your individual disease plan.

Sources: nccp

Getting the result

For each test, record its purpose, due date, result and action. Agree how results reach you and who checks a result from another hospital. If an appointment or result has not arrived, contact the named service rather than assuming no news means no action is needed.

Sources: nccn-patient

TREATMENT REFERENCE · 05

What to expect between visits

You can feel well and still need the planned checks. New symptoms deserve attention even on a day without an appointment.

Your usual pattern

Record the start and change in symptoms, temperature if advised, bowel pattern, drinking, urine and the effect on normal tasks. A simple few-line record is enough. Use it to describe a change rather than trying to grade or diagnose a toxicity yourself.

Sources: nccp

When to call

Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.

Sources: nccp

Eating, drinking and activity

Tell the team early about nausea, taste change, a sore mouth, diarrhoea or falling weight. Ask for an individual fluid and food plan if heart, kidney or other conditions limit intake. Gentle activity and planned rest can help maintain function; adjust to symptoms and ask for support when fatigue limits daily life.

Sources: nccn-patient

A held dose or changed plan

If a visit is missed or treatment is held, contact the unit for a new plan. Do not count a missed appointment as a dose received. An interval change, operation or new phase does not erase earlier immune toxicity or a permanent-stop decision.

Sources: nccp

TREATMENT REFERENCE · 06

Skin checks, sun protection and optional photographs

This complements clinical assessment and does not replace it.

Protect your skin

Use shade, protective clothing, a hat and broad-spectrum sunscreen of at least SPF 30, and avoid tanning beds. Reapply sunscreen according to its instructions. Protection matters even when it is cloudy; choose a practical approach that lets you remain active safely.

Sources: nccn-patient

Know the pattern of your skin

Ask how to examine your skin, scars and relevant lymph-node areas and how often your own team recommends doing so. A mirror or a trusted helper can make hard-to-see areas easier to check. Report a new or changing lesion, a non-healing sore or a new lump rather than waiting for a routine visit.

Sources: nccn-patient

Photographs are optional

If useful, keep a dated photograph with consistent lighting and a size reference, following your team's advice. Photographs are optional and do not diagnose a cancer or replace examination. Keep them privately; this printable passport does not require you to upload images or other personal health information.

Sources: nccn-patient

Wounds, nails and sensitive skin

Use the dressings and skin-care plan recommended for your lesion and treatment. Ask before applying new creams or remedies to an open or treated area. Worsening pain, discharge, redness, bleeding or loss of function needs advice; routine skin-care instructions are not a substitute for assessment.

Sources: nccn-patient

TREATMENT REFERENCE · 07

Pregnancy, fertility and protecting others

Discuss these precautions before treatment even if pregnancy is not currently planned.

Medicine-specific precautions

Discuss fertility before starting. Use effective contraception during treatment and for at least 5 months after the last dose. Tell the team promptly about a possible pregnancy. Discuss breastfeeding restrictions with the team before treatment; do not breastfeed without their advice. Do not stop prescribed steroids or hormone replacement yourself.

Sources: nccp

Planning ahead

Ask early about fertility preservation, sexual health and symptoms affecting intimacy. Tell the team about any chance of pregnancy, breastfeeding or a partner's pregnancy so that the correct medicine-specific advice can be given. Keep the precautions and their end dates in the written plan.

Sources: nccp

Safe handling and supply

Do not share anticancer medicines. Keep oral medicines in their original labelled packaging, away from children and anyone who could take them accidentally. Return unused medicines to the supplying pharmacy and follow its handling advice. A treatment hold does not remove pregnancy precautions.

Sources: nccp

TREATMENT REFERENCE · 08

Questions, decisions and practical support

Your priorities and the burden of treatment belong in the discussion.

Benefits and uncertainties

Ask what evidence applies to your diagnosis, stage, biomarker and treatment setting, what outcome the team hopes to improve, and what may happen without this treatment. A trial average cannot predict an individual result; a benefit measured in one setting should not be transferred to another.

Sources: nccn-patient

Alternatives and review points

Discuss other suitable treatments, clinical trials if relevant, local treatment options and symptom-focused care. Ask when the decision will be reviewed and what would prompt a change. Supportive and palliative care can help with symptoms and quality of life alongside anticancer treatment.

Sources: nccn-patient

Help with daily life

Tell the team about work, transport, caring responsibilities, cost concerns, body image, anxiety or sleep. Ask which nurse, social worker, dietitian, rehabilitation professional or support organisation can help. Use resources relevant to your country and service; a US guide may describe different access or funding.

Sources: nccn-patient

Shared understanding

Ask someone on the team to check that you can explain the medicine schedule, next tests and urgent-contact route in your own words. An interpreter, large-print copy or a trusted supporter can help; none requires you to use a smartphone.

Sources: nccn-patient

TREATMENT REFERENCE · 09

Follow-up and your continuing record

Keep this record when treatment is paused, changed or completed.

Your follow-up plan

Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.

Sources: nccn-patient

Continuing medicine precautions

Continue immune-toxicity monitoring for at least five months after the last dose, and longer when advised. Keep any prescribed steroid taper, hormone replacement and specialist follow-up. Scans follow your individual disease plan.

Sources: nccp

Treatment summary for other clinicians

Record the medicines and last dates, important reactions, any long-term hormone replacement or steroid taper, and the responsible oncology contact. Show this information in urgent care and to other specialists. A different treatment phase does not erase earlier toxicity.

Sources: nccp

The complete paper version

This handbook can be used without a phone: keep the schedule, symptom advice, medicine list, tests and contacts together. The QR code is an optional route to the matching regimen information. Ask the team to print updates when your plan changes and keep the latest dated copy.

Sources: nccn-patient

What the evidence means for your treatment

The evidence discussion must match the diagnosis, biomarker and treatment setting shown in this guide.

Evidence in advanced melanoma measures control of existing cancer; adjuvant evidence measures recurrence after surgery. Perioperative evidence concerns treatment around surgery. Ask which study and outcome match your stage and plan. These results cannot predict your individual benefit.

Sources: nccp · nccn-patient

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the treatment trying to achieve for me?

Nivolumab helps immune cells act against melanoma that cannot be completely removed or has spread. The aim is disease control; benefit and side effects are reviewed for you.

How often will I receive treatment and when does it finish?

Nivolumab every 2 weeks. Treatment continues until progression or unacceptable toxicity, with individual benefit and safety review.

What tests do I need?

Blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune conditions, transplants, previous immune reactions and all current medicines. BRAF tumour status is checked for the advanced or adjuvant melanoma indication. Blood count, kidney/liver tests and glucose before every 2-week cycle. Thyroid function is checked once a month and when clinically indicated. Cortisol and other tests are added when clinically indicated. Report new symptoms at every visit and between visits; normal blood results do not rule out immune toxicity.

What happens if treatment is paused?

If a visit is missed or treatment is held, contact the unit for a new plan. Do not count a missed appointment as a dose received. An interval change, operation or new phase does not erase earlier immune toxicity or a permanent-stop decision.

Who do I call between visits?

Ask the team to enter actual daytime and out-of-hours numbers on the wallet card. Report new symptoms promptly; do not wait for the next appointment.

> **Try explaining the plan back:** Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00483 v13a. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00483 v13a: current regimen source
- NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-06.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6498/483_Nivolumab_14-day_.pdf

https://www.nccn.org/patients/guidelines/content/PDF/melanoma-patient.pdf